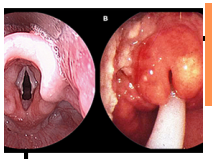
Respiratory Illnesses

Nasopharyngitis (common cold)

- Viral: congestion, runny nose, cough.
- Saline + gentle suction; fluids and rest.
- Antibiotics do not treat a cold.

Allergic Rhinitis

- Itchy nose/eyes, sneezing, clear drainage.
- Reduce triggers; prescribed antihistamine or nasal steroid.
- Usually no fever.

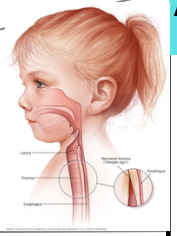


Bacterial Epiglottitis

- Drooling, tripod posture, stridor, toxic appearance.
- AIRWAY emergency: keep calm; call airway team.
- Do not inspect throat with tongue blade.

Pharyngitis (Viral or Bacterial)

- Viral: often cough/runny nose.
- Strep: fever, sore throat, tender nodes; test as ordered.
- Confirmed strep: complete antibiotics; fluids and pain relief.



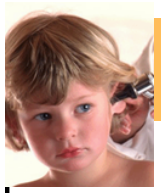
Acute Laryngotracheobronchitis (Croup)

- Barking cough, hoarseness, inspiratory stridor.
- Keep calm; corticosteroid as ordered.
- Stridor at rest/severe distress: urgent assessment; nebulized epinephrine as ordered.



Tonsillectomy

- Watch for frequent swallowing, bright-red bleeding, tachycardia.
- Report suspected hemorrhage immediately.
- Airway, hydration, prescribed analgesia; follow diet/activity instructions.



Otitis Media

- Ear pain, fever, irritability; otoscopic diagnosis.
- Pain relief; antibiotics when indicated.
- Upright feeding, smoke avoidance; reassess persistent symptoms.

COVID-19

- Fever, cough, sore throat; variable severity.
- Testing and isolation per current policy.
- Support hydration; assess breathing and oxygenation.
- Escalate distress or dehydration.

Respiratory Illnesses

Respiratory Assessment

- Rate, rhythm, effort and breath sounds.
- Retractions, flaring, grunting, stridor or wheeze.
- SpO2, color, alertness, feeding and urine output.
- Apnea, exhaustion or cyanosis = urgent escalation.

Nursing Interventions for Respiratory Problems

- Position for easier breathing; minimize distress.
- Suction nasal secretions when needed.
- Oxygen and hydration as prescribed.
- Frequent reassessment, intake/output, infection precautions.
- Avoid oral feeds if breathing makes swallowing unsafe.

Meds to Treat Respiratory Illnesses

- Albuterol: bronchospasm, not routine RSV treatment.
- Corticosteroids: asthma/croup when ordered.
- Antibiotics: bacterial infection.
- Antivirals: selected influenza/COVID cases.
- Antipyretics: weight-based orders; no aspirin for viral illness.

Pneumonia

- Fever, cough, tachypnea, crackles; possible hypoxemia.
- Support oxygenation, fluids and comfort.
- Antibiotics if bacterial; monitor work of breathing and response.

Influenza A & B

- Abrupt fever, aches, fatigue and cough.
- Fluids, rest; prescribed antiviral for eligible patients.
- Annual vaccination; respiratory precautions.

Bronchiolitis (RSV)



- Infant: congestion progresses to tachypnea, wheeze/crackles, poor feeding.
- Young infants may have apnea.
- Supportive care: suction, oxygen if hypoxemic, safe hydration.
- Bronchodilators, steroids and antibiotics are not routine.
- Monitor apnea, fatigue and intake/output.

Asthma

Cause?

- Inflammation + bronchospasm.
- Triggers: allergens, virus, exercise, smoke.

Assessment

- Wheeze, cough, prolonged expiration.
- Retractions, dyspnea, reduced activity.
- Silent chest or exhaustion can mean severe obstruction.

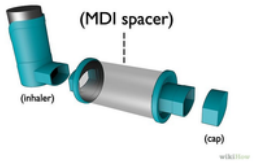
Dx

- History + variable airflow limitation.
- Spirometry when age permits.
- Peak flow tracks control; compare with personal best.

Treatment/medications

- Reliever for symptoms; controller for prevention.
- Follow the written asthma action plan.

Bronchodilators



- Albuterol relaxes airways.
- Use spacer correctly.
- Monitor tremor and fast pulse.

Anti-inflammatory

- Inhaled steroid controls inflammation.
- Use consistently; rinse mouth.
- Not a rescue drug.

Priority Education

- Teach inhaler/spacer technique and trigger reduction.
- Peak flow: green 80–100%, yellow 50–79%, red <50% of personal best.
- Follow action plan; seek urgent help for severe distress or poor response.

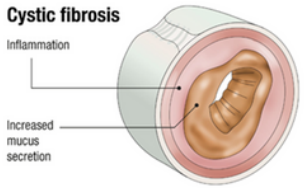
Complications

- Status asthmaticus; hypoxemia; respiratory failure. Escalate promptly.

Cystic Fibrosis

Cause?

- CFTR defect (autosomal recessive) → thick mucus, salty sweat.



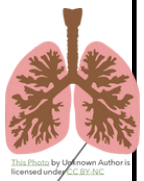
Assessment

- Chronic cough; recurrent chest infections.
- Poor growth, bulky greasy stools.
- Digital clubbing; dehydration risk.

Dx

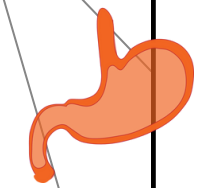
- Newborn screen prompts confirmatory testing.
- Sweat chloride test + CFTR genetic assessment.

Pulmonary



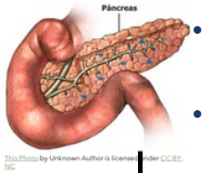
- Thick mucus obstructs airways.
- Infection and inflammation damage lungs.

GI



- Blocked pancreatic ducts → enzyme deficiency.
- Fat/protein malabsorption, steatorrhea, poor growth.

Endocrine



- Pancreatic injury can cause CF-related diabetes.
- Monitor growth and glucose as directed.

Treatment/Nursing Considerations



- Airway clearance; prescribed inhaled medicines/antibiotics.
- CFTR modulator if eligible.



- Pancreatic enzymes with every meal/snack.
- High-energy nutrition; fat-soluble vitamins and salt per plan.
- Do not crush enzyme beads.



- Insulin for CF-related diabetes as ordered.
- Coordinate glucose care with nutrition and illness needs.